

Name:	ZAHMOUL, NADIA	Admit Date:	12/10/2025
MRN:	2018366	Discharge Date:	12/10/2025
Encounter:	5001160182	Attending:	Reyes,Barron MD
DOB / Age:	4/27/1969	Copy to:	CareAware Oauth,SJMCWY
Sex / Birth Sex:	Female		

Emergency Documentation

Document Type:	ED Note Provider
Service Date/Time:	12/10/2025 18:47 MST
Result Status:	Auth (Verified)

ZAHMOUL, NADIA

DOB: 04/27/1969
Age: 56 years
Sex: Female
MRN: 2018366
Registration Date: 12/10/2025

Basic Information

Time Seen:

Reyes, Barron MD 12/10/2025 16:59

Chief Complaint

ANXIETY CRISIS. I HAVE AUTISM AND PTSD

History Of Present Illness:

56-year-old female with reported history of PTSD, autism and severe anxiety, presenting to the emergency department reporting that she has been going through a lot of stress with a legal issue and has to go back to London for a lawsuit, she has been under a lot of stress and it has caused her to have anxiety as she has run out of her benzodiazepine. She feels she is having a panic attack. She denies any fevers, cough or cold symptoms, chest pain or shortness of breath, abdominal pain, nausea vomiting or diarrhea, blood in stools or urine, dysuria or rashes, no focal numbness or weakness. She denies any suicidality or other ingestions, she does not drink alcohol, denies drug use. She is just wanting to get feeling better so she can go home and attend to her affairs. She has been hospitalized for emotional distress in the past several times. She reports that this has happened several times and she does well with a dose of Ativan and droperidol and Tylenol.

Review of Systems:

10 point review-of-systems negative except for that reviewed in the HPI.

Physical Exam

Vitals & Measurements

T: 36.5 °C (Temporal Artery) HR: 76 (Monitored) RR: 16 BP: 120/95

SpO2: 94%

HT: 157 cm WT: 48.5 kg BMI: 19.68 O2 Therapy: Room air

CONSTITUTIONAL: Patient appears that she feels unwell and anxious, tearful but otherwise nontoxic, awake and alert, appears stated age

HENT: Normocephalic, atraumatic, moist mucus membranes

EYES: Pupils are equally round, clear conjunctiva, non-icteric sclera

PULMONARY: Clear to auscultation bilaterally without wheezes, rhonchi, or rales,

Problem List/Past Medical History

Ongoing

Anemia

Chronic rhinitis

Facial pressure

Nasal septal perforation

Sjogren syndrome

Historical

Chronic sinusitis

Facial pain syndrome

Nasal septal spur

Procedure/Surgical History

- Sinus X 2

Medication Administration

Given

acetaminophen, Oral

droperidol, IV Push

LORazepam(Ativan), Oral

Allergies

No Known Allergies

Social History

Alcohol

Use:Never

Electronic Cigarette/Vaping

Electronic Cigarette Use:Never

Substance Use

Use:Never

Tobacco

Smoking tobacco use:Never tobacco user

St. John's Medical Center

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no accessory muscle use and no stridor

CARDIOVASCULAR: Regular rhythm, normal rate. No appreciated murmurs. Strong radial and posterior tibial pulses with intact distal perfusion

GASTROINTESTINAL: Soft, non-tender, non-distended, no palpable masses, no rebound or guarding, normal bowel sounds, no hepatosplenomegaly, no pulsatile masses

GENITOURINARY: No costovertebral angle tenderness to percussion

MUSCULOSKELETAL: Extremities are nontender to palpation and have no gross deformity, no edema, redness, or swelling

SKIN: Warm, dry, no jaundice/hives/petechiae/rash

NEUROLOGIC: Apart from anxiety and tearfulness patient has normal mentation and speech. Moves all extremities x 4, non-focal, ranging neck fully without a meningeal irritation

PSYCHIATRIC: Patient is very anxious, hyperventilating, tearful, constantly moving around

Family History

Healthy adult: Mother and Father.

Diagnostic Results

No qualifying data available.

ECG

Patient is declining EKG.

Medical Decision Making:

56 year old female presenting to the emergency department with reported mental health issue, stress and panic/anxiety, history of above. Reviewed patient's medical record from November 28 with renal colic, October 25 visit to urgent care with reported Sjogren's flare and sinus pressure. Upon arrival, nursing and triage notes reviewed. Patient is denying any SI or other ingestions. She appears tearful. Hyperventilating. No other chest complaints or hypoxia. Discussed differential diagnosis including but not limited to most likely acute stress response, anxiety and panic attack, need for med refill. Patient is declining any other concerning fluid loss. Discussed with the patient the risks, benefits, indications, alternatives in regards to treatment plan. At this time I have recommended that the patient have a dose of droperidol and Ativan and Tylenol per her request. We did discuss possibility of EKG to check her QT intervals and screen for any other causes for her anxiety that could potentially be cardiac related although appears low pretest probability and following her discussion the patient has declined. She appears to have clear understanding of our discussion and medical decision making capacity.

1800: Patient sleeping comfortably on monitor.

1846: Patient easily awoken and reports feeling that her symptoms are now resolved. She is very appreciative for the care. She declines any need for additional observation but does feel somewhat sleepy and we will allow her some time to rest and ensure she is steady on her feet then she reports that she will get a taxi ride home.

I discussed with the patient the importance of following up for reevaluation and to determine further care.

Return precautions discussed, verbal discharge instructions given, all questions were answered and the patient appeared comfortable and ambulatory at time of discharge home.

The above note was created using an electronic voice-to-text dictation device. Attempts were made to correct any erroneous transcriptions however an occasional dictation anomaly may persist.

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Assessment/Plan

1. Emotional crisis, acute reaction to stress | (F43.0)
2. Medication refill | (Z76.0)
Orders:
acetaminophen, 975 mg= 3 tab, Oral, Once, (Completed)
droperidol, 0.625 mg= 0.25 mL, IV Push, Once, PRN, (Completed)
LORazepam(LORazepam 1 mg oral capsule, extended release), 1 mg= 1 cap, Oral, Daily, PRN, (Ordered)
LORazepam(Ativan), 1 mg= 1 tab, Oral, Once, (Completed)
Cardiac Monitoring, 12/10/25 17:15:00 MST, Constant order, (Ordered)

Patient Discharge Condition
Stable

Discharge Disposition
Home

Patient Education

Post-Traumatic Stress Disorder, Adult
Panic Attack

Follow Up

With	When	Contact Information
Follow up with Primary Care Provider	Within 1 to 2 days	
Additional Instructions: Please follow-up with your primary care provider soon as possible. Ativan/lorazepam is a sedating medication, do not drink alcohol or take any other sedative/opioid medications with this medication as it can be life-threatening. Only take it as instructed. You may benefit from following up with a local psychiatrist to soon as possible as well as counselor. In regards to the discussion about anxiety and stresses, you may benefit from a book by Dr. David Burns called "When Panic Attacks" or "The Power of Now" by Eckhart Tolle. Please continue to explore different breathing exercises, stretching and meditation/yoga, healthy sleep habits, possible counseling/cognitive behavioral therapy and other tools to help manage your day to day stresses. At this time, it appears reasonable to return home for continued supportive care, however your situation may change and please do not hesitate to return to care at any time. Please return to the emergency department immediately for any significant worsening pain, chest pain, difficulty breathing, severe abdominal pain, bloody stools or urine, recurrent fevers, passing out, numbness/weakness, or any other concerning changes. If you are to follow-up at the local family health clinic, please call ahead to schedule your appointment. We would recommend that you contact medical records or access your online webportal to obtain copies of your medical record and/or test results from today's visit to discuss with your follow-up provider.		

Medication Reconciliation

Changed

LORazepam (LORazepam 1 mg oral capsule, extended release)1 Capsules Oral (given by mouth) every day as needed anxiety for 3 Days. Refills: 0.

Unchanged

dextroamphetamine-amphetamine (dextroamphetamine-amphetamine 10 mg oral capsule, extended release)1 Capsules Oral (given by mouth) every morning. Refills: 0.

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dextroamphetamine-amphetamine (dextroamphetamine-amphetamine 10 mg oral tablet)1 tab Oral (given by mouth) 2 times a day. Refills: 0.

estradiol (estradiol 0.025 mg/24 hours twice weekly transdermal film, extended release)Topical (on the skin) Mon/Tue/Thu/Fri/Sat.

FLUoxetine (FLUoxetine 20 mg oral capsule)1 Capsules Oral (given by mouth) every day. Refills: 0.

FLUoxetine (FLUoxetine 20 mg oral capsule)

lisdexamfetamine (Vyvanse 70 mg oral capsule)1 Capsules Oral (given by mouth) every morning.

omeprazole (omeprazole 40 mg oral delayed release capsule)1 Capsules Oral (given by mouth) every day. Refills: 0.

omeprazole (omeprazole 40 mg oral delayed release capsule)1 Capsules Oral (given by mouth) every day. Refills: 0.

progesterone (progesterone 100 mg vaginal insert)

propranolol (propranolol 40 mg oral tablet)

Electronically Signed on 12/10/2025 18:48 MST

Reyes, Barron MD